CHILDRENCARE RIDER
PLAN CODE : W25

PLAN DESCRIPTION

This is a regular premium non-participating rider which may be attached to any living and non-living Endowment or Whole Life policies (effective from 15th October 2001).

PLAN BENEFITS

1. Child Illness Benefit

In the event of the Life Assured is diagnosed as having any of the Child Illnesses listed below and the Life Assured is still alive after 30 days from the date of diagnosis of the Child Illness, Sum Assured shall be advanced in one lump sum and this Rider shall be terminated.

a. Severe Asthma
b. Bone Marrow Transplant
c. Insulin-dependent Diabetes Mellitus
d. Severe Juvenile Rheumatoid Arthritis (including Still's Disease)
e. Brain Surgery
f. Intellectual Impairment due to Accident or Sickness
g. Glomerulonephritis with Nephrotic Syndrome

A waiting period of 30 days from the date of issue of this Rider and from the date of reinstatement of this Rider is applicable for all the child illnesses stated above.

h. Leukaemia
i. Kawasaki Disease with Heart Complications
j. Rheumatic Fever with Valvular Impairment

A waiting period of 60 days from the date of issue of this Rider and from the date of reinstatement of this Rider is applicable for all the child illnesses stated above.

2. Accident Hospitalisation Benefit

a. In the event of the Life Assured confines to a Hospital for at least 12 consecutive hours and within 36 hours of an Accident, a Daily Accident Hospitalisation Benefit of RM4 per RM1,000 Sum Assured shall become payable.
b. The maximum period for which Accident Hospitalisation Benefit is payable for any injury is 180 days.
c. Accident Hospitalisation Benefit shall not be payable for Life Assured age 6 years next birthday and below.

3. Hospitalisation Benefit for Hand, Foot and Mouth Disease (HFMD)

a. In the event of the Life Assured is diagnosed of HFMD by a Consultant Physician which is medically required admission to the Intensive Care Unit (ICU) of a Registered Hospital for at least 12 consecutive hours, the Company shall pay a Daily Hospitalisation Benefit ("HFMD Hospitalisation Benefit) of 1% of the Sum Assured.
b. The HFMD Hospitalisation Benefit is subject to maximum of RM500 per day.
c. The maximum HFMD Hospitalisation Benefit is limited to a lifetime of 5 days.

PREMIUM PAYMENT

Premiums are payable throughout the term of this rider or until termination of the rider or age 25 years next birthday of the Life Assured, whichever occurs earlier. Premium may be paid annually / half yearly / quarterly / monthly in accordance with the basic policy. Premium rates for this rider are non-guaranteed and may be revised in future.

UNDERWRITING

1. Minimum / Maximum Sum Assured

Minimum : RM 10,000

Maximum : 100% of the Basic Sum Assured or RM100,000 (per life), whichever is lesser.

2. Minimum / Maximum Age at Entry

Minimum : 1 month attained age

Maximum : 16 years next birthday

3. Minimum / Maximum Term

Minimum : 9 years

Maximum : The lesser of (i) 25 – age next birthday at entry; or
(ii) term of the basic policy

4. Backdating

Backdating is allowed up to 6 months from the date of issue of the policy.

5. Premium Rates

The premium rates for ChildrenCare Rider is RM5.50 per 1000 Sum Assured for all terms and ages.

Notes:

1. The underwriting guidelines of this rider will be the same as that for juvenile plans. The rider sum assured will be subjected to the underwriting limits for juvenile plans.
2. Accident Hospitalisation Benefits is not included with other existing HB benefits for assessing HB limit for child policy.
2. Child Illnesses Benefit is not included in assessing the Living Assurance Limit.
3. When the basic policy is attached with any of the following;
   a. Payer Benefit Rider (U8)
   b. Payer Benefit with Annual Benefit Rider (U9)
   c. Payer Benefit Rider Plus (U51)
   d. Payer Benefit with Annual Benefit Rider Plus (U52)
   e. Payer Benefit Rider Plus (W21)
   f. Payer Benefit with Annual Benefit Rider Plus (W22)

   this rider sum assured is to be included in the calculation of any Payer Benefit Rider or Payer Benefit Rider Plus with/without Annual Benefit, to be waived.

ASSIGNMENT / THIRD PARTY POLICY

According to the Basic Policy.

EXCEPTIONS

The Company shall not pay the benefits for Child Illness under Plan Benefit item (1) above:

(i) for any Child Illness if there was a Pre-existing Child Illness which is the same Child Illness that is the subject of a claim under this Rider; or

(ii) for any Child Illness which existed at the date of issue of the Policy or this Rider or at the date of any reinstatement, whichever is the later; or

(iii) for any Child Illness resulting directly or indirectly from any congenital condition or inherited disorder if the age of the Life Assured at the date of the first diagnosis of the condition or disorder is 12 months or less; or

(iv) for any Child Illness which resulted from a deliberate act of the Life Assured or the Policyholder while sane or insane; or

(v) for any Child Illness which resulted from or related to HIV and/or any HIV related illness including AIDS and/or related complexes or any mutations or variations thereof.

Note: Pre-existing Child Illness shall mean a Child Illness for which prior to the issuance of this Rider or date of reinstatement of the Policy and/or this Rider:

1. *Symptoms of the Child Illness existed that would cause an ordinarily prudent person to seek diagnosis, care or treatment; or*
2. *Medical advice or treatment was recommended by or received from a Medical Practitioner.*

The Company shall not pay the benefit under Plan Benefit item (2) above if the confinement in a Medical Centre or Hospital is due directly or indirectly to:

(i) self-inflicted injuries or attempted suicide, while sane or insane; or
(ii) any Injury received:
  (a) while on duty in any navy, army, air force or police organisation; or
  (b) as a result of war, invasion, act of foreign enemy, hostilities (whether war be declared or not), civil war, rebellion, revolution, insurrection or military or usurped power, riots, civil commotion, attack by bandits or terrorists, regardless whether or not the Life Assured was actually participating; or
(iii) from engaging in aerial flights including parachuting and sky diving other than as a crew member or as a fare-paying passenger of a regular airline operating on a scheduled route; or
(iv) as a result of the Life Assured committing, attempting or provoking an assault or from any violation of the law by the Life Assured; or
(v) while under the influence of alcohol or drugs; or
(vi) from engaging in racing on horse or wheels; or
(vii) from engaging in any underwater activity.

DEFINITION OF CHILD ILLNESSES

1. Severe Asthma

"Severe Asthma" as evidenced by either

(i) An acute attack of severe asthma leading to admission to a Medical Centre or Hospital and assisted ventilation with a mechanical ventilator for a continuous period of at least four (4) hours to establish control of the asthma attack on the advice of a consultant paediatrician; or
(ii) At least three (3) of the following features of chronic, severe asthma:
  (a) Continuous daily usage of oral corticosteroids for a minimum period of six (6) months on the advice of a consultant paediatrician to control the child's asthma.
  (b) The presence of Harrison's sulcus chest deformity as confirmed by a consultant paediatrician.
  (c) Significant growth impairment attributed by a consultant paediatrician to the child's asthma (which is for this purpose defined as a height below the third percentile for the child's age and sex in a child with asthma whose height has previously been recorded at or above the fifth percentile at a routine developmental examination at the age of at least one year).
  (d) An average of at least three (3) admissions to Medical Centre or Hospital per year in the previous two years to control acute attacks of asthma. An admission to Medical Centre or Hospital will be counted for this purpose if it results in a stay in Medical Centre or Hospital of at least two (2) nights' duration on the advice of a consultant paediatrician.
  (e) Significant and persistent limitation of the peak expiratory flow rate (which is for this purpose defined as maximum peak expiratory flow rate recordings of less than 80% of the rate predicted for a child of the same age, sex and build while taking the treatment prescribed by a consultant paediatrician for asthma). The recordings are to be made by a consultant paediatrician on at least four occasions at intervals of no less than one (1) month in a period of at least twelve (12) months. The paediatrician certifying the recordings should be satisfied that the child is complying with optimal prescribed asthma medication throughout the period to which the recordings relate.

2. **Leukaemia**
   Unequivocal diagnosis of Leukaemia, confirmed by histology, requiring chemotherapy and/or radiotherapy treatment.

3. **Bone Marrow Transplant**
   The actual undergoing of or confirmation of acceptance on to the official Malaysia waiting list for a Medically Necessary transplant of bone marrow from a human donor as a recipient.

4. **Insulin-dependent Diabetes Mellitus**
   Insulin dependent diabetes mellitus as characterised by the continuous dependence on exogenous insulin for the preservation of life as diagnosed by a consultant endocrinologist. Evidence of dependence for a minimum of six (6) months will be required before a claim is considered.

5. **Rheumatic Fever with Valvular Impairment**
   A confirmed diagnosis by a consultant paediatrician acceptable to the Company of acute rheumatic fever according to the revised Jones criteria for its diagnosis. There must be involvement of one (1) or more heart valves and at least mild valve incompetence attributable to rheumatic fever as confirmed by quantitative investigations of the valve function by a qualified cardiologist acceptable to the Company.

6. **Kawasaki Disease with Heart Complications**
   The diagnosis of Kawasaki disease by a consultant paediatrician or cardiologist. There must be echocardiographic evidence of cardiac involvement manifested by dilation or aneurysm formation in the coronary arteries present for at least six (6) months after the initial acute episode.

7. **Severe Juvenile Rheumatoid Arthritis (including Still's Disease)**
   Severe juvenile rheumatoid arthritis as diagnosed by a consultant rheumatologist, with widespread joint destruction and major clinical deformity of at least three (3) or more of the following joint areas: hands, wrists, elbows, knees, hips, ankle, cervical spine or metatarsophalangeal joints in the feet. The symptoms of arthritis should have persisted for at least one (1) year.

8. **Brain Surgery**
   The actual undergoing of Medically Necessary surgery to the brain under general anesthesia during which a craniotomy is performed. Bur hole and brain surgery as a result of an accident is excluded.

9. **Intellectual Impairment due to Accident or Sickness**
   Significant and Permanent Neurological Impairment or Significant and Permanent Loss of Intellectual Capacity as diagnosed by a consultant paediatrician or consultant neurologist as a direct result of accident or sickness.
   The Neurological Impairment or Loss of Intellectual Capacity must be confirmed for the Life Assured who is under the age of six (6) by a Denver Development Screening Test, Singapore (DDST, Singapore) to be performed in Malaysia with a reading of "Abnormal", or for the Life Assured aged six (6) or above, by an IQ score of less than 70.
   The Significant Neurological Impairment or Significant Loss of Intellectual Capacity must result in the need for special child care or special schooling as certified by a consultant pediatrician or consultant neurologist and must persist for at least six (6) months before a claim will be paid. No claim will be paid until the Life Assured is aged four (4) or above.
   There must be unequivocal proof that the Significant and Permanent Neurological Impairment or Significant and Permanent Loss of Intellectual Capacity is a direct result of an accident sustained or sickness suffered by the Life Assured after the risk commencement date of this Rider.

10. **Glomerulonephritis with Nephrotic Syndrome**
    Glomerulonephritis with nephrotic syndrome which has continued for a period of at least six (6) months, with or without intervening periods of remission. The diagnosis of

nephrotic syndrome must be made by a qualified paediatrician acceptable to the Company and who should confirm that a treatment regimen appropriate to the clinical presentation has been followed throughout the period to which the syndrome relates.